Audio file

chapter 16 voiceover full.mp4

Transcript

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So we are going over Chapter 16, which is the assessment of the eyes.

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A couple of things we will go over in this chapter, we will go over some eye disorders.

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We'll also go over some eye assessments and some vision assessments as well.

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So here are the objectives.

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So what is the structure and the function of the eyes?

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What do we need to differentiate between normal and abnormal findings of the eyes and our vision?

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and analyze interview and physical assessment data related to the eyes and vision to formulate any clinical judgments.

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So again, we always go through our pathophysiology and kind of what the different components of the external eye will look like.

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So there is an eyelid and the eyelashes.

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There are two types of muscles.

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There's the striated type of muscle and the smooth muscle.

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What the function of the eyelids do is protect our eyes from any foreign bodies that we might encounter.

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And also, it limits the amount of light that can enter the eyes.

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So it can prevent us from having any issues with vision if we are seeing bright light and things like that.

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And then the eyelashes.

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What the eyelashes do is they filter dust and dirt from the air that enters the eyes.

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So that's just a little bit of a review of your external eye structures.

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So we talked about the external structures.

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Now let's consider the internal structures.

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So again, this is just review on anatomy.

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So there are a couple components of the internal eyes include the sclera and the cornea, the iris, the lens, the retina, and the optic disc.

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So now let's kind of talk about each of those components.

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So the sclera and the cornea, think of it as like a white protective covering.

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that physically supports the internal structures of the eyes.

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So we might call it the window of the eye.

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What the cornea does is it permits kind of that entrance of light to the eyes, which passes through the lens into the retina.

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And then the iris.

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So the iris is that circular disc of muscle

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which kind of helps with our PERLA.

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So PERLA is pupils or equal, round, reactive to light and accommodation.

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We'll talk about that further in the slides.

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But usually what the iris is, since it's a disc of muscle, it adjusts the amount of response to pupil size.

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So say, for example, you take a pen like

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and you put the pen light and assess the pupils, what that will do is the pupils will constrict or will get smaller.

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And that is kind of what the iris kind of does.

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Also with the muscle fibers, I would like to add the muscle fibers of the iris also decreases the size of the pupil to accommodate for near vision.

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and dilate for when there's far vision that is necessary.

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So that is your iris.

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I'd also like to add the iris is kind of where eye color is and it contains pigment that can help us that helps determine the eye color.

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Also, there's certain variations of the iris depending on what ethnicity the client may be and things like that.

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So sometimes people of Northern European and Scandinavian descent are historically known to have blue eyes, while Asian and African descent have brown eyes.

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So sometimes, depending on ethnicity, the color will change.

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And then the lens.

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So the lens is a biconvex.

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So think about two transparent structure that is kind of located to the back of the iris.

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And what it does is it refracts light or bends light into light rays into the retina.

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The retina is that innermost layer.

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So as we keep continuing to go into the internal portion of our eye structure, that retina is that innermost layer.

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So the lens bends light rays into the retina, which then the retina receives

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and sends that visual stimuli to our brain for us to see, for us to achieve vision or kind of look at our daily surroundings.

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And then the optic disc is a cream colored kind of circular area of the eye.

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Usually that is where the optic nerve enters kind of the eyeballs.

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So this is the optic disc.

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You guys will see this is kind of that cream colored circular area that has optic nerves where that kind of enters in the eyes.

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Generally that will be kind of toward the medial or nasal side of the eye.

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So this is what a good or a normal optic disc will look like.

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So let's talk about visual fields and pathways.

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So

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Visual field is what refers to what a person can see in one eye.

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When we talk about visual fields, visual fields are broken up into four different quadrants.

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So the upper temporal, so think about kind of the forehead, the lower temporal, the upper nasal, and the lower nasal, kind of where your nose might be.

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So there's four quadrants.

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So when we're thinking about our visual fields,

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It kind of results in when we have vision it results in binocular vision or two-eyed vision.

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So what happens is when we get that response from the brain to stimulate vision that cortex fuses two different images or two images to provide

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kind of three-dimensional vision.

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So think of it like binocular vision.

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So what is visual perception?

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So visual perception occurs as light rays strike the retina where they are transformed into nerve impulses

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conducted to the brain through the optic nerve and then interpreted.

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From there, light must pass through the cornea before reaching that retina.

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So that's kind of what that visual perception is.

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So think about visual perception simply put as light rays are striking the retina

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which are now transformed into that nerve impulse or that signal, conducted to the brain through the optic nerve and then interpreted as what we see as vision.

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And then visual reflexes.

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There's two types of reflexes, a pupillary light reflex and accommodation.

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You guys will kind of practice this in lab as well.

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So visual reflexes

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is the ability of the pupils to constrict when exposed to bright light.

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So again, visual reflexes is the light reflex that causes the pupils to constrict when we show bright light or shine bright light in the eyes.

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So usually, it's a direct reflex with light shown.

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And that's a normal response.

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So for example, if I take a pen light and shine a light in my patient's pupils and the pupils constrict, that is a normal response to bright light and that is normal pupillary light reflex.

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When we think of accommodation, accommodation is another functional reflex that you guys will kind of do in your head to toe as well.

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How you assess accommodation.

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So accommodation is kind of seeing if the eyes converge towards the middle or converge towards your nose and constrict at the same time.

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What you're assessing there is that reflex to focus on near objects through the movement of the different muscles.

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So how we test accommodation.

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is you're going to have a client or a patient look at a specific item or area in the room.

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Say, for example, have them look at a clock right in front of them.

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And then what you're going to do is you're going to go right in front of them with a pen light.

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And then what you're going to do is instruct the patient to let you know when they see the pen light as you proceed to get it closer to their nose.

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Okay, what we should be seeing with normal accommodation, okay, is the eye should be converging towards kind of towards the, towards our nose, okay, and it's constricting at the same time.

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Okay, so that is accommodation.

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This is just a visual for you guys to kind of understand some of the things that we kind of talked about, and it kind of makes it a little bit simpler.

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If you guys like to have any visuals and things like that, you guys can use the chart.

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So now let's look at our collection of subjective data.

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So what is the data we might be looking at when we're assessing our eyes or our patient's eyes?

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So vision changes, that's the first thing I'm gonna add.

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So when we're thinking about vision changes, a lot of vision changes can be related to acute problems, okay?

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So if a patient has head trauma or increase intracranial pressure after a stroke or after different types of traumatic experiences, okay?

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So we gotta think about what vision changes are occurring.

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Also, what vision changes are related to aging, right?

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So your macular degeneration or cataracts.

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Okay.

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Diabetes.

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So sometimes there's a disorder that can occur if diabetes progresses further, which can cause blindness.

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Okay.

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Hypertension.

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If a person's high blood pressure.

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or if a person's blood pressure is too high, that it can cause vision changes, right, or neurologic disorders.

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Second thing is spots or floaters.

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So spots or floaters are seen in clients with myopia or clients that are over the age of 40.

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So they're kind of black, small spots that kind of float around in their eyes.

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Okay.

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And then halos and rings.

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Halos and rings sometimes is associated with narrow angle glaucoma.

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And sometimes when we think about halos and rings, we also need to think about what are they seeing and how is it related.

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So you guys will see the different types of eye disorders further on in this PowerPoint slide.

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And then you guys also have swelling and redness.

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So this is really important to understand if we're trying to assess if there's any abnormalities, okay?

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So usually there shouldn't be any swelling or redness in the eyes or the eyelids.

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However, sometimes you might see that with some inflammatory infections if there's a sty in somebody's eye.

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And things like that.

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So when we're assessing that we're also assessing if there's any pain or discharge.

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Okay.

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Sometimes pain and discharge can occur if a foreign body was, you know, comes in the eye and it can kind of cause that irritation.

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Usually, most eye disorders are not associated with pain.

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So if there is some sort of eye pain, it is suggested that we seek help if needed, especially in that situation.

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Again, discharge, if there's any white discharges coming from the eye and they can't open their eyes, could be related to a bacterial infection.

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So

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Also, corrective lenses.

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Do people wear glasses or contacts?

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So if you're maybe doing some sort of vision testing, do they need to wear their glasses or contacts at that time?

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And then we always talk about our safety, especially with our sun safety.

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So always make sure that when we ask our patients, are they wearing sunglasses?

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Because sometimes if it's not appropriate, sunglasses can cause cataracts.

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Okay.

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Think about also their diet.

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Okay.

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Sometimes there's increased risk.

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So depending on what diet the patient is, there could be increased risk for eye disorders.

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And then smoking.

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Smoking has been found.

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to actually increase the chance of cataracts.

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So some of these things we might ask our clients, if we're doing admission assessment and they have a history of eye disorders, what type of eye drops they take for kind of all those things.

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So think about these types of data that we are collecting.

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So our older adult considerations.

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So vision is one of the most common things

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that starts to deteriorate as we take care of geriatric clients.

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So our big thing that we have to consider is are they safe at home?

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Are they safe to perform their activities of daily living?

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So going up the stairs, cooking for themselves, taking care of their pets if they have pets at home.

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Because if they might not be safe at home because they're having double vision or they're having

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floaters or they're kind of having blurred vision.

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Are they safe to go up the stairs or drive?

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Secondly, when was the last eye exam?

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A lot of older adults still have licenses and they still drive even though their eyes are not in the best condition.

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That can cause a lot of issues.

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Is there any loss of central vision that might impair them to kind of do their daily activities?

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Or any visual aids?

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Do they use any glasses to read?

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Are they readers or do they wear glasses all the time?

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Usually what we kind of ask these for our older adults is we kind of think about

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still, you know, having normal vision.

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If not, can you tell me what you're kind of kind of seeing?

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Are you nearsighted?

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Are you farsighted?

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Are you starting to lose any central vision to where you might see a black spot in the middle of your vision that's impacting you?

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Okay.

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Think about also glaucoma for older adults.

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What are some risk factors for glaucoma?

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So age older than 40,

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A previous family history.

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Usually African Americans are at increased risk for glaucoma.

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So especially open angle glaucoma, African Americans are at higher risk.

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East Asian individuals are usually at risk for closed angle glaucoma.

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Diabetes, high blood pressure, taking any corticosteroids, any type of steroid.

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or previous past eye injuries.

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So think about those things with the older adults.

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So now let's talk about the inspection process.

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So when we inspect, always remember, right, we're inspecting, we're looking at kind of the eye structures.

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We're assessing there's any redness, any swelling, any discharge or any lesions, right?

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There should know there should for your normal

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inspection of your external structure.

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There should be no redness in this while I know discharge or any lesions.

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Right?

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Those are all abnormal.

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Sometimes there could be a redness or a crusting that is kind of along the lids or eyelids that could suggest

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Blepharitis, we'll talk a little bit more about that, which is that infection caused by staph aureus.

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The lids should be able to close open, should be able to close and open and should not be stuck together.

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With wide discharge, that is not normal.

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So when we're assessing the general position of the eyebrows, eyelids, and lashes, have maybe the patient keep their head straight,

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while looking from side to side up to the ceiling to also assess that muscle movement.

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Also look at the eyeballs.

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Is it protruding?

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So are the eyeballs enlarged?

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Are they protruding or does it have a sunken appearance?

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Also the thing I like to kind of inspect with my external structures is I like to look at the white part of the eye to see if there's any changes.

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So I like to kind of look at the sclera.

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of the eye to make sure that there's no liver issues going on because patients who are African American will present with liver failure later on in the disease process and sometimes a yellow or jaundice coloration will show up in their sclera and also in their skin so that's also one indication that there could be something abnormal.

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Next, we're going to inspect the anterior structures, so the cornea and the lens and the iris and the pupil.

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So when we think about the inspection process, look at the shape and the color of the iris and the shape of the pupil.

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Is it round?

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Does it constrict or does it not constrict?

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when light is shine or shown into the eyes.

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So what we need to think about is when we're doing that pupillary light reflex.

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So what are we doing?

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What are we doing when we're assessing for pupillary light reflex?

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We are putting a pen light or shining a pen light in somebody's eyes

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A nursing consideration I always like to say is please warn them before you shine a light in their eyes.

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You can say, okay, I'm going to shine a bright light in your eyes.

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I'm just going to let you know I'm going to do it so it doesn't scare you.

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So shine a light in their eyes and make sure each pupil constricts.

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On that photo you guys see on the left side, that is a pupil gauge that is usually kind of

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put in or kind of it's part of your pen light.

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So there's that little gauge that is part of your pen light.

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And what you're assessing there is the numbers that is the size of the pupil.

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So say for example that you shine a light in your eyes and one of the eyes is constricting but the other is not.

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That could be related to maybe a neurological disorder.

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If patients are in comas, sometimes it doesn't shine or it doesn't actually constrict.

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So we got to think about that as well.

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So more in detail about pupillary light reflex.

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So remember I mentioned a light reflex is a pupil response that should constrict when exposed to bright light.

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This can be seen as a direct reflex or an indirect consensual reflex, in which the exposure to light in one eye results in constriction of the pupil in the opposite eye.

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So when you shine a light in one eye, it's going to constrict, and it should be consensual or the same.

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And if you shine the light in the right eye, you shine the light in the right eye, it should constrict.

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Shine a light in the left eye, it should constrict.

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So the people should always be consensual and they should have a response that is equal

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on each eye.

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If it doesn't, there could be a neurological issue.

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If you guys sometimes look at the way your pupils change in response to light, depending on what types of light you may be in, you will see different sizes of the pupil that may change.

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So when you kind of see not even when light is being shown in the eyes, but when lighting may change in different rooms, you might see bright light, that constriction, normal light, kind of that normal pupil size, where it's not as constricted in dim light, it might be a little bit bigger, depending on how our vision can kind of compensate for that.

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But make sure you guys just understand that pupillary light reflex, if it is normal, it should have a consensual reflex.

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So if you shine a light in one eye, it should constrict, that's equal.

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And you shine a light in the left eye, it should constrict.

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And right and left eye should both be equal with your pupillary light response.

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So collecting your objective data.

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So this is now we're going to do-- we're going to talk about PERLA.

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So PERLA, you guys will hear nurses say PERLA is present.

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So pupils are round.

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They're reactive to light and accommodation.

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So we assess PERLA by taking a pen light, shining a light in our patient's pupils.

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And what we should see is both pupils should constrict in response to bright light.

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And then your accommodation.

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Accommodation is kind of that kind of muscle movement that I was telling you guys about.

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So what you're going to have the client do is stare at a far object or a distant object.

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And they're going to put a near object.

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You can use your finger or you can use your pen line.

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Either or will work.

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You're going to put your pen light, you're going to kind of proceed to put the pen light closer to the patient's nose.

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And you're going to instruct the client, tell me when you see the pen or tell me when you see the pen light coming towards your nose.

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What you're going to be assessing there is do the pupils constrict in response to that test and do they converge or kind of the pupils go towards the nose.

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That is a normal pupillary response if you guys are assessing for accommodation.

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Just remember PERLA, pupils round reactive to light and accommodation.

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A normal pupillary response is the pupils should constrict in response to bright light and should be consensual and equal.

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You guys have probably seen or done a Snellen chart in school or trying to get a driver's license.

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So what the Snellen chart does is it evaluates your visual acuity.

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So how well is your vision?

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Usually the procedure for this is you have the patient stand 20 feet from the actual picture.

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And then what you instruct your patient to do is to read the lowest level that they can possible and usually depending on that patients get like a 20 out of 20 and depending on the score they will they're testing that visual acuity.

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Okay.

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Again, you can have them read like the longest or the farthest line possible.

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Read the alphabet and they usually should be within 20 feet from the chart and have them cover one eye.

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So you guys are kind of over the snellen, so I'm not going to go too much into the snellen chart.

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So six cardinal fields of gays.

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This is the thing I commonly do.

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with my patients when I am assessing kind of muscle innervation, so muscle movement and muscle strength, also to assess cranial nerves three, four, and six.

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We'll talk more about the cranial nerves in the neuro chapter, but I'll just keep it simple for this chapter.

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So when we're assessing the six cardinal fields of gaze, we have the client

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keep their head straight and we are going to take our finger and have them follow our finger in different positions.

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So six different positions.

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Okay, so what we're gonna have to do with them is ask them, okay, can you move your eyes and follow my finger to the right, follow my finger to the left.

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Great.

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And the next thing you're gonna have them, can you follow my finger down, follow my finger up,

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And then you're going to do the same thing to the left side of the eye.

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Can you follow my finger up and follow my finger down?

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So that's kind of your six cardinal fields of gaze.

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So you're going to go right to left, follow my finger back and forth.

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Look up and down to the right, look up and down to the left.

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And that's your six cardinal fields of gaze.

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So usually what you're looking for is that the eye movement should be smooth.

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symmetrical through the six directions that the eye is moving.

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If during that assessment you find that the six cardinal fields of gaze is something called dystagmus.

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So that is when the eye movement is kind of staggering and shaking in between those movements.

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That is abnormal, that is not normal.

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This could be seen with Ms.

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different types of brain lesions,

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Patients that use narcotics, so think about like a fine oscillating movement, so like a staggering type of movement and doesn't move slowly.

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Okay?

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So the confrontation field test.

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You guys will realize this is a lot more common exam that you will do as a nurse because this is a type of visual kind of vision evaluation I have done.

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As a nurse when I took primarily when I primarily took care of patients that had strokes.

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So when I think of patients that had strokes and there's vision loss or peripheral vision loss, this is the test I would do.

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So you guys can see it assesses visual fields for gross kind of that loss and peripheral vision.

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So how do we do this?

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Okay, how do we do the confrontation field test?

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So to perform the confrontation test, you're gonna place yourself two feet away from the client, usually at eye level.

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You're gonna instruct your client to cover the left eye, cover the left eye while you cover your right eye.

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Look directly at the client with your uncovered eyes, okay?

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Next, what you're gonna do is fully extend your left arm at the midline, so towards the middle of the nose, okay, and slowly move your finger, okay?

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until the client sees your finger.

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So what you're assessing is when do they see my finger and is there any issues with peripheral vision if they cannot see it.

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Then you're going to test four other quadrants.

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You're going to test four visual quadrants for this.

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Next you're going to test three more of those visual fields of the opposite eye and kind of do the same.

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So that is your confrontation test.

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All you're doing here is assessing your peripheral vision and making sure that they don't have any peripheral vision loss.

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So, for example, your normal kind of evaluation is the client should see the finger that you're putting or the pencil you're putting at the same time that your finger is going through that area of peripheral vision.

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Say, for example, that there is a delayed or there's an absent reaction.

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Okay, it could be someone that had a stroke and they're having that peripheral vision loss.

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So older adult considerations.

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So when we're thinking about internal and external structures or vision, what are some of the things we might see?

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So central acuity might decrease.

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So central acuity has a chance of decreasing, so their loss of central vision

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could be cloudy, it could be more blurred, so we have to think about that.

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This is very common, so the skin around the eyes will start to sag or wrinkle.

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Eyes have less shine, they're a little more dull.

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There might be more chance of a cloudy cornea, which you might see maybe with cataracts, right?

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There's also something called arcus senellus.

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Arcus senellus

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is a grayer white arc.

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I have a photo of it right there.

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It's kind of around the eyes due to an increased buildup of lipids.

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So it does not really impair vision, though.

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And then something called entropion, ectropion, and xanthelasma.

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So xanthelasma are raised or yellow plaques that are usually located near the inner canthus.

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Again, can be seen with high lipid levels, so cholesterol.

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Entropion is an inverted lower eyelid, so the eyelid is actually inverted down.

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It can cause pain or injury due to the lower lashes rubbing against the conjunctiva and cornea.

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And then there's ectropion, which is the averted lower eyelid, and that results in exposure and drying of that conjunctiva.

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And there's some terms.

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There's photophobia, which is the inability to tolerate light, so photomphobia.

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Diplopia, which is double vision.

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Myopia, which is impaired far vision.

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Presbyopia is impaired near vision, which could occur after 45.

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Scotoma is just your blind spots.

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Strabismus is a misalignment of the eyes.

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So there's generally an issue with the cranial nerves and there's poor muscle control.

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There could be farsightedness associated with it or brain and nerve problems.

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And then we talked about nystagmus, which is the oscillating movement.

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So that staggering kind of movement that's not smooth and symmetrical.

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That is when we're doing the six cardinal fields of gaze, right?

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Usually with Ms.

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brain lesions, or narcotic use.

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We're just going to go over some eye diseases on the upcoming slides.

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So macular degeneration.

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Macular degeneration is one of the most common kind of vision loss disorders you guys will see with older adults.

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Generally what happens is as individuals age, sometimes there is

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a deterioration of that retina.

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So what happens when the retina deteriorates is there is a major loss of that central vision.

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So you guys can see in this photo that is normal vision.

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Macular degeneration is like a blind spot or blurred vision that's in the very middle of their visual field.

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Usually it's one of the

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kind of most leading cause of vision loss in Americans over 60 years of age.

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So when we're thinking about risk factors, especially for macular degeneration, smoking, remember we talked about smoking being a big cause for some visual, female is more common, prolonged sun exposure without any kind of our sun safety, light eye color, high fat, high cholesterol diet,

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high sugar diet, hypertension, cardiovascular disease.

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So when we're kind of educating patients as they age, maybe think about educating them on a kind of healthy balanced diet, limiting high cholesterol foods, exercising regularly and always wearing sunglasses when they go out and doing regular eye exams.

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So always just remember with your macular degeneration, it's that deterioration

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of that retina and they have that major loss of that central vision that is either blurred or it's a blind spot.

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Cataracts, so cataracts again commonly seen with our kind of aging adults.

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as well.

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It's the leading cause of blindness, which is generally preventable.

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Usually you can do this is preventable.

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You can get cataract surgery to help relieve or to help this kind of visual issue.

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Okay.

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Remember, cataracts does not generally deal with a loss of central vision.

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However, it will cause a cloudy kind of very

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cloudy view of their vision.

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So generally most common is age 65.

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So people that are 65 and above are increased risk for getting cataracts, also smoking, increasing age, diabetes, excessive exposure to sunlight.

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You guys will see this is one of the most common ones as well.

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high blood pressure, previous eye injury or inflammation.

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Okay.

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Prolonged use of steroid medication can cause cataracts.

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Okay.

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Usually some symptoms you might see with cataracts.

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There's a cloudy film over kind of their their vision.

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Usually their colors are a little bit more faded.

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They're more sensitive to light and glare and sometimes see halos or little rings and they might possibly also have double vision as well.

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So glaucoma, glaucoma, there's open angle glaucoma and narrow angle glaucoma.

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I just want you guys to, we're just gonna kind of talk about regular or just kind of talk about the overall basics of glaucoma.

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So the second leading cause of blindness.

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So when we think about glaucoma, right?

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So think about the difference between cataracts, macular degeneration, and now glaucoma.

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So glaucoma is the second leading cause of blindness.

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It is an actually, it's caused with deterioration or damage to the optic nerve.

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Usually if there's a high, abnormally high amounts of pressure within the eye, so intracranial pressure as well.

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Glaucoma is irreversible.

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So once it occurs, it can cause blindness.

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Usually there's a peripheral loss of vision, so they might not have good peripheral vision.

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Usually screening is really, really important to make sure that we detect these differences in pressure really early on.

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Usually patients I've taken care of a glaucoma have certain eye drops that their physicians prescribe to them and it does help slow that progression.

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I will say the one kind of population that's usually at risk for glaucoma is African Americans.

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Usually it's a lot more common than these, you know, in that ethnicity.

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So usually they should be tested for glaucoma every one to two years.

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So just some abnormalities of the external eye.

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These are just photos.

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I'll kind of tell you guys kind of what is happening in each photo.

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So this is Ectropion.

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So we kind of talked a little bit about the older adult considerations, right?

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And what we might be seeing.

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So Ectropion is that outwardly turned out outwardly turned lower lid, sorry.

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So you guys can see that that lower lid is kind of flipped outward and that could cause maybe some dryness and that conjunctiva as well.

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You can see it's kind of exposing that lower lid.

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Entropion you guys can see instead of kind of outward it is inward.

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So entropion it's the lower lid is kind of turned into itself so inside the eye itself.

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This is called pitosis.

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Pitosis is a drooping of the upper eyelid.

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So you guys can see the upper eyelid has a little bit of a droop.

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So when you guys are assessing patients that might be having a stroke, please look at the history because sometimes this could look like stroke symptoms.

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So I always like to educate myself to make sure that this is normal for this patient to have pitosis.

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So pitosis is that drooping upper eyelid.

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You guys can see the difference between that right and left eye on that image.

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So when we talked about our considerations of kind of the eyes, we talked about do they have any protruding eyeballs or kind of that retracted eyelid?

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This is something called expothalamus.

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So someone has hyperthyroidism or they have Graves' disease, this is a very common kind of like eye, external eye issue that occurs, very common.

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in this disease process, it's the eyeballs are protruding so they're popped out a little bit and then the eyelids are retracted so they're kind of pushed up kind of towards the kind of the upper part towards the eyebrows so they're kind of pushed a little bit more back and then the eyeballs itself are more protruded and enlarged and they kind of bulge out a little bit more so that's exothalamus usually seen with hyperthyroidism.

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So blepharitis, you guys can see the redness, that's just inflammation of the eyelids.

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Usually related to staphylococcus aureus, so that's some sort of infection with kind of the eyes there, so it could be a little bit painful for this client.

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So you guys just think about blepharitis as inflammation related to staph aureus.

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This is a hortilium or something called a stye.

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So I'm sure that's one of the most common things that you might see if there's any sort of infection.

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So this is a little bit different than blepharitis because blepharitis is kind of the infection of the eyelid itself.

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With your hortilium or your stye, this is acute.

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This will kind of last a very short amount of time and usually it's a localized staph infection.

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Usually the hair follicles kind of at different areas of the eye and it's usually only in one specific spot at a time.

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So that's a hortilium.

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And this is conjunctivitis, most commonly known as pink eye.

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It's a generalized inflammation of the conjunctiva, so the kind of white part of our eyes.

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You guys can see it's kind of reddish around that area.

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Conjunctivitis is just pink eye.

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Jaundice just means yellowing of the sclera.

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So that white part of our eyes gets kind of that yellowish color.

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Usually when clients are having liver failure and their liver is not releasing the toxins in the body, what happens is there is a buildup of bilirubin which can show up in the skin.

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With your kind of more African-American patients, it might show up in the maybe the

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on their actual part of their hand that's not the back of their hand.

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It might show up kind of on their palms of their hand.

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It's usually more of a yellowish pigment.

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Usually you can see that it's very yellow and you will not miss a patient that has jaundice because it's very

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easy to spot.

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So you guys can see that's what it might look like.

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A yellow color due to buildup of bilirubin that is not becoming processed in that liver.

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So you guys can see that yellowish color.

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Here's a review question.

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So you're assessing a client, people are reflex.

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They want to chart the nurse client reflex was normal.

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What will they document?

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Both pupils dilate in response to bright light.

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Both pupils dilate when converging to focus on light.

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Both pupils constrict in response to bright light.

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And both pupils constrict when focusing on a distant object.

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So I want you guys to kind of think about this and think about what the answer might be.

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When we think about pupil light reflex, what is your normal

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response, right?

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It is when both pupils are constricting in response to bright light.

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That second option would be is it's a distractor.

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That is not correct, right?

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Then both pupils constrict when focusing on a distant object that could be part of a combination.